

ABNORMAL LABOUR

1. Difficult labour is termed as.....

A. Cystocia

B. Dystocia

C. Eutocia

D. Extocia

Answer: B. Dystocia

Rationale:

Dystocia means abnormal or difficult labour due to problems with power, passenger, or passage.

Eutocia = normal labour.

Cystocia / Extocia are not correct obstetric terms.

2. What is the leading indication for primary cesarean delivery?

A. Age over 40 years

B. Dystocia

C. Full bladder

D. Preeclampsia

Answer: B. Dystocia

Rationale:

Failure to progress (dystocia) is the most common reason for first-time cesarean section.

Age, full bladder, and preeclampsia alone are not leading causes.

3. Which of the following is the most accurate predictor of a successful vaginal delivery?

A. Clinical assessment

B. Clinical assessment and pelvimetry

C. Computed tomography measurements of the bony pelvis

D. Progress of descent of presenting part and cervical dilatation

Answer: D. Progress of descent of presenting part and cervical dilatation

Rationale:

Real-time labour progress reflects the actual ability of the pelvis to accommodate the fetus.

Pelvimetry (including CT) does not reliably predict labour outcome.

4. Arrest disorders can occur during which phase of labor?

- A. Active phase
- B. Either latent or active phase
- C. Latent phase
- D. None of the above

Answer: A. Active phase

Rationale:

Arrest disorders are diagnosed only in the active phase when adequate contractions fail to produce cervical dilatation or descent.

5. All of the following cause labour dystocia EXCEPT:

- A. Shoulder dystocia

- B. Occipito-anterior
- C. Occipito-posterior
- D. Malpresentation

Answer: B. Occipito-anterior

Rationale:

Occipito-anterior is the most favorable position for labour.

Shoulder dystocia, malpresentation, and occipito-posterior cause difficult labour.

6. Which of the following is FALSE about face presentation?

- A. Brow presentation most of the time changes to face presentation
- B. Decsent mechanism is complete different from vertex presentation
- C. Delivery is possible if mentum appears beneath the symphysis pubis
- D. This is rare presentation above inlet and the denominator is the mentum

Answer: B. Descent mechanism is completely different from vertex

Rationale:

Although modified, the mechanism of labour is similar, not completely different.

Mentum is the denominator and vaginal delivery is possible if mentum is anterior.

7. Which of the following is TRUE about the management of the first stage of labour disorders?

- A. Arrest disorder with adequate contractions and possible C – section
- B. Prolonged latent phase as a result of false labour is managed by sedation and observation
- C. Protraction disorder of the active phase is managed by augmentation with amniotomy or oxytocin
- D. All of the above

Answer: D. All of the above

Rationale:

Arrest with adequate contractions → C-section

False labour → sedation and observation

Protraction → augmentation with oxytocin or amniotomy

8. You perform vaginal examination for a 35 years old G1P0 client in labour. Foetal nose and mouth are palpable. The chin is pointing toward the maternal left hip. This is:

- A. Brow presentation
- B. Mentum transverse position
- C. Occiput transverse position
- D. Transverse lie

Answer: B. Mentum transverse position

Rationale:

Palpable mouth and nose = face presentation

Chin toward left hip = mentum transverse

9. What are the maternal causes of malpresentation? I. Contracted pelvis II. Pelvic tumour III. Mullerian abnormality IV. Multiparity

- A. I, II, & III
- B. I, II & IV
- C. I, III & IV

D. II, III & IV

Answer: B. I, II & IV

Rationale:

Contracted pelvis, pelvic tumor, and multiparity distort uterine shape.

Mullerian anomalies are less common maternal causes.

10. Which of the following does NOT predisposes to the incidence of breech presentation?

A. Eclampsia

B. Multiparity

C. Oligohydramnios

D. Primigravida

Answer: A. Eclampsia

Rationale:

Breech is associated with multiparity, oligohydramnios, prematurity, not eclampsia.

11. Classifications of breech presentation are done under two main groups namely:

- A. Complete and incomplete
- B. Extended and frank
- C. Flexed and frank
- D. Footling and knee

Answer: A. Complete and incomplete

Rationale:

Breech is broadly divided into complete and incomplete (frank, footling).

12. All the following aid with the diagnosis of breech presentation EXCEPT:

- A. Buttocks felt at pelvic pelvic region
- B. Head felt at the fundus
- C. Longitudinal lie
- D. Mouth and nose felt per vaginum

Answer: D. Mouth and nose felt per vaginum

Rationale:

Mouth and nose indicate face presentation, not breech.

13. Among these is NOT a foetal complication of breech presentation.

- A. Cord prolapse
- B. Foetal hypoxia
- C. Intracranial injury
- D. Prolonged labour

Answer: D. Prolonged labour

Rationale:

Prolonged labour is mainly a maternal complication.

Cord prolapse, hypoxia, and intracranial injury affect the fetus.

14. In which fetal presentation vaginal delivery can be expected?

- A. Brow presentation
- B. Face presentation when chin lies direct to the sacrum
- C. Face presentation with the chin under the symphysis pubis
- D. Shoulder presentation

Answer: C. Face presentation with chin under symphysis pubis

Rationale:

Only mentum anterior face presentations can deliver vaginally.

15. What is the presenting diameter in the face presentation?

- A. Occipito frontal
- B. Occipito posterior
- C. Submento bregmatic
- D. Suboccipito bregmatic

Answer: C. Submento-bregmatic

Rationale:

This is the smallest diameter when the chin is anterior, allowing vaginal delivery.

16. All of the following cause labor dystocia EXCEPT:

- A. Face presentation
- B. Occipito anterior
- C. Occipito posterior
- D. Ovarian mass

Answer: B. Occipito anterior

Rationale:

Occipito-anterior facilitates labour, not obstructs it.

17. Techniques of delivery for breech presentation include all of the following EXCEPT:

- A. Burns-Marshall method
- B. Lovset's maneuver